

Before You See Your Doctor

A worksheet to fill in by hand and bring to your appointment — second edition, July 2026

Your doctor fills out the medical report, called the DS2444B. It is long, and it asks for detail that is hard to reconstruct from clinic notes in a short appointment. This worksheet is for you to fill in beforehand, in your own handwriting, and hand to your doctor at the start of the visit.

It is not a form the government issues and it is not sent anywhere. It exists so that the person who knows your daily life best, which is you, can give your doctor the raw material, and so your doctor can spend the appointment on clinical judgement instead of on reconstructing your history.

Write plainly, and do not tidy it up

The most useful answers are ordinary and specific. “I nearly dropped my coffee cup because my arm gave out” is worth more than “I have weakness in my arm.” Describe your worst days as well as your ordinary ones. Nothing here is complaining, and nothing here is too small.

Do not leave anything out because you think it is unrelated

Conditions that seem minor on their own can matter a great deal when added together. The assessment looks at your combined situation, not one diagnosis at a time.

If printing all of this is a problem

It is nineteen pages, and printing costs money. If you need to print less, the pages that do the most work are section 4, which is one page per condition, section 7 on pain and endurance, and the last page, which is the note for your doctor. Everything else can be written on any paper you have. The layout is a convenience, not a requirement.

You do not have to finish this in one sitting

It is long on purpose. Do a section, put it down, come back. If your hands or eyes make writing hard, someone else can write while you talk, and that is perfectly acceptable. Skip anything that does not apply to you.

1. Your details

Your name

Date of birth

Appointment date

Family doctor

Seeing this doctor since

Specialists you see

2. What you most want your doctor to remember today

One or two paragraphs. If your doctor read nothing else on this sheet, what would you want them to know?

3. All of your conditions, at a glance

List everything, including conditions you have had for years and ones that come and go. Photocopy this page if you run out of rows.

Diagnosis	Year	Which specialist	Current treatment	Stable or worsening

Why the last column matters

Worsening is a finding. If a condition has been getting worse, say so and say over what period. A condition described as stable can be read as manageable, which is not the same thing as bearable.

4. One page for each condition

This is the most useful part of the worksheet. Fill one out for each condition that affects your daily life. Two blank copies follow. Photocopy the page before you write on it if you need more, or continue on separate paper.

Condition

What it feels like, and what it does

Your symptoms, in your own words.

Test results or findings, if you know them

Scans, bloodwork, specialist findings. Leave blank if you are not sure.

How it affects your daily life

Ordinary things. Getting dressed, cooking, stairs, holding a cup, following a conversation.

What you can no longer do, or can only do with difficulty

What happens if you push past your limits

This is one of the most important questions on the sheet. What does it cost you the same day, and the next day? How long does it take to recover?

How often the symptoms happen

How many days in a typical week or month. How long each episode lasts. Whether you can tell one is coming.

Has treatment helped, and by how much

“A little,” “not at all” and “it made things worse” are all useful answers.

One page for each condition (continued)

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5. Medications and treatments you take now

Include prescriptions, over the counter, supplements, creams and injections. The side effects column matters as much as the rest.

Medication or treatment	What it is for	Does it help	Side effects

Side effects are evidence, not complaints

Someone who cannot tolerate a treatment is not someone refusing it. If a medication caused stomach bleeding, made you sleep all day, or made your symptoms worse, write it down. That is part of the clinical picture and it protects you later.

6. Treatments you have tried over the years

Tick everything that applies, then explain below.

- | | | |
|--|---|---|
| ☐ Family doctor | ☐ Specialist | ☐ Surgery |
| ☐ Physiotherapy | ☐ Occupational therapy | ☐ Counselling or therapy |
| ☐ Pain clinic | ☐ Injections | ☐ Rehabilitation program |
| ☐ Chiropractic or massage | ☐ Medication trials | ☐ Other |

What has been tried, and why significant limitations remain despite it. Include anything stopped, refused, or never started, and the reason.

If you turned down a treatment, write down why

Declining a treatment can be read badly later by someone who was not in the room, as though help was refused. If a specialist raised concerns about the risks, or the cost or travel put it out of reach, or a waitlist is years long, or your condition itself makes it hard to keep up with, say so. Ask your doctor to record the reason in the report. Same facts, very different reading.

7. Pain, fatigue and how long you can keep going

Fill in both columns. This is the single most important change on this page. One middling number hides the thing that matters most, which is how much you swing.

	On a bad day	On an ordinary day
Pain, from 0 to 10		
Fatigue, from 0 to 10		
How long you can sit, in minutes		
How long you can stand, in minutes		
How long you can walk, in minutes		
How much you can lift, in pounds		
Hours of sleep you get		
How many hours you are up and out of bed		

Write the bad-day figure in the first column and the ordinary-day figure in the second. If it changes hour to hour, say so in the space below.

How many bad days do you have?

In a typical week or month. How long they last. Whether you can tell one is coming. “Three bad days most weeks, sometimes a whole week” tells the story. “It varies” does not.

Unpredictability is the case, not a detail

A person who is reliably limited can often be accommodated. A person who cannot know what any given morning will bring cannot be, and that difference is what this assessment exists to capture. If you can never plan a day, that is the most important sentence you will write.

8. What you have difficulty with

Tick anything you find hard, unsafe, or cannot do at all. There is space underneath to explain any of them.

- | | | |
|---|--|--|
| ☐ Walking | ☐ Standing | ☐ Sitting for long |
| ☐ Stairs | ☐ Lifting | ☐ Carrying |
| ☐ Reaching | ☐ Using your hands | ☐ Bending or kneeling |
| ☐ Driving | ☐ Getting to appointments | ☐ Shopping |
| ☐ Cooking | ☐ Cleaning | ☐ Laundry |
| ☐ Bathing | ☐ Dressing | ☐ Managing money |
| ☐ Memory | ☐ Concentrating | ☐ Following instructions |
| ☐ Finding your words | ☐ Reading or screens | ☐ Managing stress |
| ☐ Sleeping | ☐ Eating or swallowing | ☐ Being around people |
| ☐ Getting up on time | ☐ Being somewhere on time | ☐ Getting through a whole day |

9. What a bad day looks like

Walk through one from waking up to going to bed. What goes wrong, what you cannot finish, what you need help with, how many times you have to stop or lie down.

10. What an ordinary day looks like

Not your best day. An average one. The difference between this and the last section is part of what the report has to capture.

11. Things you used to do and cannot do now

Work, housework, cooking, driving, shopping, hobbies, seeing people, caring for others. Say roughly when each one stopped.

What you used to do	When it stopped	What stops you now

12. Help you need from another person

Anything someone else does for you, or that you cannot do alone or safely alone. Include help you would need but do not have.

13. Equipment and aids you use

Canes, walkers, braces, wheelchairs, hearing aids, lifeline, shower chair, anything else. Include anything you are waiting on.

14. Work history, and what ended it

What you did, for how long, and what happened. If you tried to keep working and could not, describe what that cost you. If you tried to retrain or go back to school, say what happened.

15. How your conditions together affect working

Tick anything that describes your experience, then explain underneath.

- | | |
|--|--|
| ☐ I cannot be sure I would get there on time | ☐ I would need to stop and rest without warning |
| ☐ I cannot predict how I will be from one day to the next | ☐ Pain slows me down or stops me |
| ☐ Fatigue means I cannot keep going | ☐ My body will not let me do the physical parts |
| ☐ My memory or concentration is not reliable | ☐ My mental health symptoms get in the way |
| ☐ I could not keep up full-time hours | ☐ I could not keep up part-time hours reliably either |

In your own words: what happens when you try to keep to a schedule, and what has happened the last time you tried?

Describe, do not conclude

Write what happens to you, not what you think the decision should be. “I tried to keep a volunteer shift last spring and after two weeks I was in bed for a month” carries far more weight than “I am unemployable.” The first one is evidence only you can give. The second one is the assessor's job, and saying it for them does not help you.

16. Anything else

Anything that has not fit above. Falls, hospital visits, pain at night, mood, or how you manage on your own.

A note for your doctor

Hand this page over with the completed worksheet

Your patient is applying, or reapplying, for Assured Income for the Severely Handicapped (AISH), and the form you are being asked to complete is the DS2444B medical report. The preceding pages were filled in at home, to save you reconstructing daily function during the appointment.

The determination turns on whether there is a severe and permanent impairment that substantially limits the ability to earn a livelihood. Four things carry disproportionate weight and are commonly under-recorded:

Variability, recorded as a range

Where function fluctuates, the range is the finding. A rating written as severe on flare days and mild between carries the file; a blended middle rating erases the evidence. Frequency, duration and predictability of episodes are load-bearing. Section 7 of the worksheet records bad-day and ordinary-day figures separately for this reason.

Permanence, and whether options are exhausted

Files are commonly refused on the theory that further treatment might restore capacity. Where that is clinically not the case, stating it plainly closes the question. Where a treatment is nominally available but inaccessible within a clinically relevant timeframe, that is worth recording as such.

Response to exertion

Where a patient reports disproportionate symptom exacerbation following activity, the recovery period is often more probative than the activity tolerance itself. A patient who can perform a task once but is incapacitated afterward does not have that capacity in any employment-relevant sense.

Accommodation

Language describing what accommodations would enable can read as capable with support. Where it is clinically true that no accommodation renders sustained competitive employment reliable, saying so directly is the clearer finding. An employer can adapt a workstation; none can adapt to unpredictable incapacity.

Where adherence has been limited, the reason belongs on the record. An unexplained note of non-compliance can undermine an otherwise strong file, whereas cost, side effects, access, intolerance, or the disability itself impairing self-management are findings in their own right.

Please complete the report to the true clinical picture and no further. Accuracy is the position that withstands review. This worksheet supplies context, not conclusions, and nothing in it should displace

your own assessment.

A fuller question-by-question guide to the DS2444B is available free at albertadisabilitysystembreakdown.netlify.app. The official form is at open.alberta.ca.

The Alberta Disability System Breakdown — albertadisabilitybreakdown@outlook.com — general information, not legal or medical advice. Free to share, print and post. Second edition, built from two versions contributed by members of the group.